

Forensic Data Recovery and Cross-Jurisdictional Legal Defense Strategy: Medical Records Extraction for the Dual National

Executive Strategic Overview

This operational report articulates a high-precision, forensic data retrieval strategy designed for a dual British-Irish national currently resident in London. The primary objective is to secure the release of comprehensive, unredacted medical and psychiatric records from two distinct legal and administrative jurisdictions: **Northern Ireland**, specifically the Belfast Health and Social Care Trust (Royal Victoria Hospital, Acute Mental Health Inpatient Unit Ward 5), and the **Republic of Ireland**, specifically the Health Service Executive (HSE) West (University College Hospital Galway / Saolta University Health Care Group).

The Data Subject faces a complex medico-legal challenge involving administrative or disciplinary actions where the defense predicates on proving that alleged behavioral misconducts were manifestations of documented disabilities, specifically **Traumatic Brain Injury (TBI)** and **Complex Post-Traumatic Stress Disorder (C-PTSD)**. To substantiate this defense, the Data Subject requires a granular, longitudinal medical history that correlates acute psychiatric episodes with administrative timelines. However, the standard provision of medical records—often delivered as disorganized, photocopied bundles or flattened, non-searchable PDF images—creates an insurmountable barrier for a TBI survivor suffering from executive function deficits and working memory impairments.

This analysis establishes that the refusal to provide data in **machine-readable, digital formats** (e.g., native .PST for communications, .XML/.JSON for audit logs, and OCR-enabled PDFs for clinical notes) constitutes a violation of the **Disability Discrimination Act 1995 (Northern Ireland)** and the **Equal Status Acts 2000–2018 (Republic of Ireland)**. The report leverages the Data Subject's technical knowledge of hospital IT infrastructure—specifically the **Epic 'Encompass' system** recently deployed in Belfast ¹ and the **National Integrated Medical Imaging System (NIMIS)** in Ireland ²—to dismantle institutional defenses claiming "disproportionate effort."

Furthermore, the strategy preemptively neutralizes the high probability of data suppression under the "Serious Harm" exemptions (GDPR Art. 23 / DPA 2018 Sch 3) by activating a **clinical proxy mechanism** within the initial request. This ensures that even if the Data Subject is deemed "too vulnerable" to view their own notes, the chain of evidence is preserved through a nominated London-based physician, maintaining the integrity of the defense while satisfying duty-of-care obligations.

Part I: Jurisdictional Legal Frameworks: A Comparative Analysis

The Data Subject operates across two distinct but overlapping legal regimes. While the **General Data Protection Regulation (GDPR)** provides a harmonized baseline for data access rights across Europe, the post-Brexit landscape in the UK and the specific national implementation statutes create a bifurcated legal environment. Understanding the nuances between the **Data Protection Act 2018 (UK)** and the **Data Protection Act 2018 (Ireland)** is critical to the success of the requests, as the enforcement mechanisms, supplementary rights, and disability statutes differ fundamentally.

1.1 The Northern Ireland Framework (Belfast Health & Social Care Trust)

The Belfast Health and Social Care Trust (BHSCT) functions as a Data Controller subject to the UK GDPR and the Data Protection Act 2018 (UK). The legal architecture in Northern Ireland is unique within the UK due to the retention of specific pre-2010 equality legislation.

1.1.1 The Right of Access and the "Format" Mandate

Under **Article 15 of the UK GDPR**, the Data Subject has an absolute right to obtain a copy of personal data undergoing processing. However, the critical leverage lies in **Article 12(1)**, which requires the controller to take appropriate measures to provide information in a "concise, transparent, intelligible, and easily accessible form." Furthermore, **Article 15(3)** explicitly states that where the data subject makes the request by electronic means, the information shall be provided in a "commonly used electronic form" unless otherwise requested.

The Belfast Trust typically defaults to providing paper copies or scanned images on a CD. For a TBI survivor, a "scanned image" is not an "accessible form" because it cannot be searched, indexed, or processed by assistive technology (screen readers). The request must therefore cite Article 12(1) in conjunction with the **Disability Discrimination Act 1995** to argue that a "flattened PDF" is legally insufficient.

1.1.2 The Disability Discrimination Act 1995 (DDA)

Unlike Great Britain, which adopted the Equality Act 2010, Northern Ireland retains the **Disability Discrimination Act 1995 (DDA)**. This distinction is vital for citation accuracy.

- **Sections 19–21 (Goods, Facilities, Services):** The Trust has a statutory duty to make adjustments where a practice, policy, or procedure makes it "impossible or unreasonably difficult" for disabled persons to make use of a service.³
- **The Anticipatory Duty:** The duty to make reasonable adjustments is anticipatory, not reactive. The Trust cannot claim surprise or lack of technical capability when requested to provide accessible formats; they are legally required to have anticipated the needs of

disabled patients. Failure to have a system ready to export digital records for a TBI patient constitutes a systemic failure of this duty.

- **Intersection with Data Protection:** The Information Commissioner's Office (ICO) has explicitly stated that organizations must consider reasonable adjustments when responding to Subject Access Requests (SARs). A refusal to format data accessibly is a simultaneous breach of the DPA 2018 and the DDA 1995.³

1.2 The Republic of Ireland Framework (HSE West / Saolta Group)

University Hospital Galway (UCHG) and the Health Service Executive (HSE) are subject to the EU GDPR and the Data Protection Act 2018 (Ireland).

1.2.1 Statutory Instruments and Health Data

The access to health data in Ireland is governed by specific secondary legislation that modifies the GDPR baseline.

- **S.I. No. 121 of 2022 (Data Protection Act 2018 (Access Modification) (Health) Regulations 2022):** This Statutory Instrument is the primary mechanism for restricting access. It mandates that if a controller believes access would cause "serious harm" to the physical or mental health of the data subject, they cannot simply refuse the request. Instead, they must consult a health professional.⁴
- **The Proxy Mechanism (Regulation 9):** Crucially, Regulation 9 of S.I. 121/2022 provides that if access is refused, the controller must inform the data subject that they can nominate a health professional to receive the data. This creates a procedural "safety valve" that the Data Subject can exploit preemptively.
- **Discretionary Consultation:** The 2022 Regulations updated the 1989 Regulations by making the consultation with a health professional discretionary for non-health controllers, but for a hospital (a health services provider), the consultation remains a standard requirement before refusal.⁵

1.2.2 The Reasonable Accommodation Mandate (Equal Status Acts 2000–2018)

Ireland does not have a "Reasonable Adjustment" duty under that specific name; instead, it uses the concept of "**Reasonable Accommodation**" under the **Equal Status Acts 2000–2018**.

- **Section 4 (Reasonable Accommodation):** A failure to do all that is reasonable to accommodate the needs of a person with a disability involves discrimination if it makes it "impossible or unduly difficult" for the person to avail of the service.³
- **The "Nano Nagle" Precedent:** The Supreme Court decision in *Nano Nagle School v Daly* radically redefined the scope of "reasonable accommodation" in Ireland. While an employment case, its principles regarding the extent of the duty extend to service provision. The Court held that a "wise employer" (or service provider) must engage in meaningful consultation and consider all appropriate measures—including the redistribution of tasks or, in this context, the formatting of data—unless it imposes a disproportionate burden.³

- **Application to SARs:** The HSE cannot merely provide paper records if doing so makes the data "unduly difficult" for a TBI survivor to process. The *Nano Nagle* precedent implies they must actively reformat the data (e.g., via Optical Character Recognition or digital export) to render it accessible, as long as the cost is not prohibitive. Given that modern IT systems like NIMIS and IPMS have native export functions, the "cost" is negligible.

1.3 Comparative Jurisdictional Table

The following table synthesizes the divergence in legal instruments and technical environments between the two target jurisdictions.

Feature	Belfast Health & Social Care Trust (NI)	UCHG / HSE West (ROI)
Data Protection Statute	UK GDPR / DPA 2018 (UK)	EU GDPR / DPA 2018 (Ireland)
Disability Statute	Disability Discrimination Act 1995 (DDA)	Equal Status Acts 2000–2018
Accommodation Duty	"Reasonable Adjustment" (Section 21)	"Reasonable Accommodation" (Section 4)
Legal Threshold	"Impossible or Unreasonably Difficult"	"Impossible or Unduly Difficult"
Key Case Law	<i>Archibald v Fife Council</i> (UK HL)	<i>Nano Nagle School v Daly</i> (IESC 63)
Health Data Exemption	DPA 2018 Sch 3, Part 2 ("Serious Harm")	S.I. No. 121 of 2022 (Health Regulations)
Electronic Health Record	Epic (Encompass) - Live June 2024	NIMIS (Radiology) / IPMS (Admin) / Paper
Regulator	ICO (Northern Ireland Office)	Data Protection Commission (DPC)
Audit Capabilities	Granular "Break the Glass" Logs (Epic)	Limited / System-Specific (IPMS)

Part II: The Neuro-Legal Defense: TBI, C-PTSD, and the Right to Accessible Data

The success of these requests hinges on transforming the Data Subject's medical condition from a background bio-psychosocial fact into a binding legal constraint on the hospitals' administrative procedures. The "Standard Operating Procedure" for SARs—photocopying paper charts and scanning them into flattened, non-searchable PDF images—must be framed not just as inconvenient, but as legally discriminatory.

2.1 The Neurobiology of Access: Traumatic Brain Injury (TBI)

Traumatic Brain Injury results in specific cognitive deficits that fundamentally alter how an individual interacts with information.

- **Executive Function Deficit:** TBI survivors often struggle with sequencing, planning, and organizing complex information. A disorganized pile of 500 paper pages requires the patient to physically sequence and organize the data—a task their disability specifically impairs.
- **Working Memory Impairment:** The "scratchpad" of the brain is damaged. A TBI survivor cannot "hold" a date from Page 1 in their mind while flipping to Page 50 to check a medication. They require external memory aids to perform this cross-referencing.
- **The "Digital Prosthetic" Argument:** Digital search functions (Ctrl+F), metadata sorting (by Date, Author), and screen readers act as "cognitive prosthetics." They offload the working memory requirement onto the computer. By refusing to provide searchable data, the hospital is effectively confiscating the patient's "prosthetic" and demanding they run on a broken leg.
- **Legal Implication:** Under the DDA 1995 (NI) and Equal Status Acts (ROI), providing non-searchable data renders the service "functionally inaccessible." It places the disabled person at a "substantial disadvantage" compared to a non-disabled person who might retain the cognitive capacity to sort through paper files.

2.2 Complex Post-Traumatic Stress Disorder (C-PTSD) and Safety

The content of the requested data—records of involuntary admission, restraint, sedation, and "Notes of Concern"—is inherently traumatic.

- **The Trigger Mechanism:** For a C-PTSD sufferer, physical contact with the "trauma object" (the physical paper file) or the requirement to attend the "trauma site" (the hospital) to view records can trigger dissociation or panic.
- **Remote Digital Access as a Safety Measure:** Requesting secure digital delivery is not a luxury; it is a clinical safety measure. It allows the Data Subject to process the information in a controlled, safe environment (their home), utilizing text-to-speech or other distancing tools to manage emotional regulation.
- **Standard Practice Challenge:** Hospitals often insist on "viewing in person" for sensitive mental health records to "support" the patient. The request must argue that for this specific patient, such a requirement is counter-therapeutic and discriminatory.

Part III: Forensic Architecture: Northern Ireland (Belfast Trust & Encompass)

The Belfast Health and Social Care Trust recently underwent a monumental digital transformation that provides the Data Subject with a powerful forensic lever: the **Encompass** system.

3.1 The "Encompass" (Epic) Transformation

On **June 6, 2024**, the Belfast Trust went live with **Encompass**, a localized implementation of the **Epic** Electronic Health Record (EHR) system.¹ This creates a "bifurcated" record that requires a dual-strategy request.

3.1.1 Post-June 2024 Data: "Born Digital"

All clinical interactions, nursing notes, medication administration records (MARs), and mental health detention forms created after June 6, 2024, are "born digital." They reside in the Epic database (built on InterSystems Caché).

- **Native Export Capability:** Epic has a built-in "Release of Information" (ROI) module designed to export patient records to PDF or machine-readable formats. There is *zero* need for printing and scanning.
- **The "My Care" Portal Limitation:** The Trust offers a patient portal called "My Care" (Epic MyChart).⁶ While useful, it is **forensically insufficient**. It filters out "sensitive" notes, psychotherapy details, and third-party data. The SAR must explicitly state: *"I require the full Legal Health Record (LHR) export from the Epic backend, not the filtered view available via My Care."*

3.1.2 Pre-June 2024 Data: The Legacy & Migration

Data prior to June 2024 exists in legacy systems (e.g., PAS, localized mental health databases) or physical paper charts.

- **Migration Reality:** When Encompass went live, the Trust migrated a "focused set" of data (demographics, active meds, allergies).⁷ The detailed historical notes were likely *not* migrated into Epic's active fields but may be accessible via a "Media" tab or legacy viewer.
- **Forensic Demand:** The request must ask for *"All legacy records, including those not migrated to active fields in Encompass, provided as OCR-scanned PDFs."*

3.2 Audit Trails and the "Break the Glass" Log

In a dispute involving potential maladministration or "whistleblowing retaliation" (as hinted in the user's context), the **Audit Log** is often more valuable than the medical notes themselves.

- **Epic's Granularity:** Epic tracks every single user interaction. It logs who opened the chart, which specific tab they viewed (e.g., "Psychiatry" vs. "Demographics"), and how long they stayed.
- **"Break the Glass":** If a patient is a VIP or staff member (or flagged as sensitive), Epic requires a user to enter a reason for access ("Break the Glass").
- **The Request:** The Data Subject must specifically request: *"The full 'Access Log' or 'Audit Trail' for my Electronic Health Record in Encompass, specifically detailing the User ID, Date/Time, and Department of every access event from June 2024 to present."*
- **Legal Basis:** This is personal data (info about the security of the account) and is accessible under GDPR, distinct from the health data itself.⁸

Part IV: Forensic Architecture: Republic of Ireland (HSE West, Saolta, & NIMIS)

The landscape in Galway is defined by fragmentation. The Data Subject is not dealing with a single monolithic entity like the Belfast Trust, but a complex web of overlapping agencies.

4.1 The Saolta vs. CHO 2 Structural "Black Hole"

A critical failure point for SARs in the West of Ireland is the split between Acute and Community services.

- **Saolta University Health Care Group:** This entity manages **University Hospital Galway (UCHG)**, including the Emergency Department (ED) and physical medicine wards.
- **Community Healthcare Organisation 2 (CHO 2 / Community Healthcare West):** This entity manages **Mental Health Services**, even when those services are physically located *inside* the hospital building (e.g., the Acute Mental Health Unit).⁹
- **The Risk:** If the Data Subject sends a SAR to "University Hospital Galway," the Saolta FOI officer may check the *medical* file, find no psychiatric notes (because they belong to CHO 2), and close the request.
- **The Strategy:** The request must be **dual-addressed** and explicitly link the two entities. *"I am directing this request to BOTH the Saolta Group (for medical/ED records) and CHO 2 (for Psychiatric/Mental Health records) to prevent the fragmentation of my file."*

4.2 NIMIS: The Radiology Lever

The Republic of Ireland uses the **National Integrated Medical Imaging System (NIMIS)**, a centralized PACS system for storing X-rays, MRIs, and CT scans.²

- **The "Print" Trap:** Hospitals often try to provide X-rays printed on paper or film. For a TBI survivor needing to consult with a neuro-specialist in London, a paper print is diagnostically useless.
- **The DICOM Standard:** The request must demand the images in their native **DICOM (Digital Imaging and Communications in Medicine)** format. This format preserves the high resolution and metadata (radiation dose, machine settings).
- **Delivery Mechanism:** NIMIS allows for the burning of CDs/USBs or secure electronic transfer. The request should specify: *"I require the raw DICOM files, not JPEGs or PDF reports."*

4.3 Mental Health Records: The "Kardex"

Unlike Belfast's Epic system, Irish mental health units often still rely on physical paper charts known as the "**Kardex**" for nursing notes.

- **Forensic Implication:** These are handwritten.
- **Reasonable Accommodation:** The Data Subject cannot search handwritten notes. The

request must demand: *"Where records exist only in handwritten format, I request a typed transcript OR a high-resolution scan that has been processed with Optical Character Recognition (OCR) to the best technical standard available, as a reasonable accommodation for my visual/cognitive processing deficits."*

Part V: The "Serious Harm" Exemption and the Clinical Proxy Protocol

Both jurisdictions allow data to be withheld if it causes "Serious Harm" to mental health. Given the Data Subject's history (Acute Ward 5, UCHG Psych Unit), the Trusts will almost certainly invoke this exemption to withhold raw clinical discussions regarding involuntary detention or "risk to others."

5.1 The Legal Mechanism of Suppression

- **Northern Ireland (DPA 2018 Sch 3, Part 2):** A controller can withhold health data if an "appropriate health professional" certifies it would likely cause serious harm to the physical or mental health of the data subject or another person.¹¹
- **Republic of Ireland (S.I. 121/2022):** Similar provision, but explicitly requires the controller to *offer* access to a nominated health professional if direct access is refused.⁴

5.2 The "Nominated Health Professional" Strategy

The user must not wait for the refusal. The strategy is to **preemptively nominate** a trusted clinical proxy.

- **The Workflow:**
 1. **Identify a London-based GP or Psychiatrist** willing to receive the file.
 2. **Embed the Nomination:** The initial SAR letter must contain a clause: *"If you intend to rely on the Serious Harm exemption to withhold any portion of my record, I immediately nominate Dr. [Name] to receive the unredacted data on my behalf."*
 3. **The Legal Trap:** If the hospital refuses the patient *and* refuses the doctor, they are acting unlawfully. By nominating the doctor upfront, the Data Subject forces the hospital to either release the data or admit they are obstructing access without a legal basis.

Part VI: Operational Strategy: Overcoming Bureaucratic Fragmentation

6.1 Identity Verification (The "Ping Pong" Delay)

Hospitals frequently delay remote SARs by rejecting scanned IDs for "security reasons."

- **The Fix:** The Data Subject is in London. They must obtain a **Certified Copy** of their Passport, signed and stamped by a practicing Solicitor or Notary Public in London.
- **Forensic Weight:** A solicitor-certified document carries legal weight. Rejecting it implies the hospital is accusing a London solicitor of fraud. This stops the delay tactics immediately.

6.2 Escalation Pathways

- **Northern Ireland:** If the Belfast Trust refuses the digital format, the complaint goes to the **Information Commissioner's Office (ICO) Northern Ireland**. The specific ground is "Failure to comply with Article 12(1) GDPR and Section 21 DDA 1995."
- **Republic of Ireland:** If UCHG/CHO 2 refuses, the complaint goes to the **Data Protection Commission (DPC)**. The specific ground is "Failure to provide Reasonable Accommodation under the Equal Status Acts and S.I. 121/2022."

Part VII: Draft Forensic Data Request Instruments (Annotated)

These drafts are engineered to be "too dangerous to ignore." They combine legal citation, technical specificity (Encompass/NIMIS terms), and disability rights into a cohesive demand.

7.1 Draft 1: Belfast Health & Social Care Trust (Northern Ireland)

Subject: URGENT: Subject Access Request (UK GDPR) & Reasonable Adjustment Request (DDA 1995) – – DOB:

To: Medical Legal Services (MedLegalServs@belfasttrust.hscni.net)

Cc: Data Protection Officer (DataProtection@belfasttrust.hscni.net); Public Liaison (publicliaison@belfasttrust.hscni.net)

Date:

Re: Request for Medical Records (Ward 5, RVH) & Encompass Audit Logs

Dear Medical Legal Team,

I am writing to formally make a **Subject Access Request (SAR)** under **Article 15 of the UK GDPR** and the **Data Protection Act 2018** for my personal data held by the Belfast Health and Social Care Trust.

1. Identity & Verification

I am a former patient of the Royal Victoria Hospital (Acute Mental Health Inpatient Unit, Ward 5). I am currently resident in London. Please find attached a **Solicitor-Certified Copy of my Passport** to verify my identity. Given my dual nationality and residence, any requirement for in-person verification would be an unreasonable barrier.

2. Request for Reasonable Adjustment (Disability Discrimination Act 1995)

I formally disclose that I am a person with a disability as defined under the **Disability Discrimination Act 1995 (DDA)**, specifically **Traumatic Brain Injury (TBI)** and **Complex**

Post-Traumatic Stress Disorder (C-PTSD).

- **Cognitive Barrier:** My TBI results in significant impairments in executive function, working memory, and information processing. The provision of large volumes of paper records or "flattened" (non-searchable) PDF images renders the data functionally inaccessible to me, as I cannot utilize assistive technology (screen readers, keyword search) to sequence and process the information.
- **Legal Duty:** Under **Section 21 of the DDA 1995**, the Trust has a statutory duty to make adjustments where a practice (such as providing paper copies) makes it "impossible or unreasonably difficult" for a disabled person to make use of a service.
- **Adjustment Required:** I specifically request that all records be provided in **digital, machine-readable formats** (OCR-enabled PDF, Native Digital Export). I explicitly **refuse** paper copies or image-only scans.

3. Scope of Request

I request the following specific datasets relating to my admissions, specifically my time in Ward 5, RVH:

A. Clinical Records (Encompass & Legacy):

- **Post-June 2024 (Encompass/Epic):** I am aware the Trust migrated to the **Encompass (Epic)** system on June 6, 2024. For all data generated after this date, I require a full **"Release of Information" export** from the Epic system, to include Clinical Notes, Nursing Notes, Medication Administration Records (MAR), and Care Plans. Do *not* refer me to the "My Care" portal, as I require the full Legal Health Record, not the filtered patient view.
- **Legacy Records (Pre-June 2024):** All scanned notes from my time in Ward 5, provided as **OCR-Searchable PDFs**.
- **MDT Notes:** Minutes of all Multidisciplinary Team meetings where my care, discharge, or detention was discussed.
- **Mental Health Orders:** Copies of all statutory forms (Form 2, Form 3, etc.) related to any detention under the Mental Health (NI) Order 1986.

B. Incident & Safety Reports (Datix):

- All incident reports filed on the **Datix** (or equivalent risk management) system regarding me, including any reports of restraint, seclusion, or "challenging behavior." I understand these are often stored separately from the clinical file.

C. Forensic Audit Trails (The "Access Log"):

- I request a copy of the **Access Log** (Audit Trail) for my electronic health record.
- **Specific Query:** Please list all staff members who have accessed my record from ** to [Present]**, detailing the **User ID**, **Date/Time**, and **Department** of the access event.
- **Justification:** This request is made to verify the integrity of my data processing and ensure no unauthorized access occurred, which is my right under GDPR Recital 63.

4. Serious Harm Test (Schedule 3, DPA 2018)

I anticipate you may consider withholding portions of my psychiatric notes under the "Serious Harm" exemption (Schedule 3, Part 2, DPA 2018).

- **Nominated Health Professional:** To prevent refusal or delay, if you determine that direct access would cause serious harm, I hereby authorize and nominate the following

health professional to receive the unredacted data on my behalf immediately:

- **Name:** Dr. [Name of GP/Psychiatrist]
- **GMC Number:** [Number]
- **Address:** [Practice Address in London]
- **Email:** [Official Practice Email]

5. Preservation of Data

I request that a **Legal Hold** be placed on my data immediately to prevent any automated deletion, particularly of audit log metadata, pending the resolution of this matter.

I look forward to your acknowledgment within 48 hours and a full response within one calendar month.

Yours sincerely,

[Address]

[Phone]

7.2 Draft 2: HSE West / Saolta University Health Care Group (Republic of Ireland)

Subject: Subject Access Request (GDPR Art. 15) & Reasonable Accommodation Request (Equal Status Acts) – – DOB:

To:

1. **Freedom of Information / Data Protection Officer**, University Hospital Galway / Saolta Group (foi@universityofgalway.ie / uugh.foi@hse.ie)
2. **Deputy Data Protection Officer**, Community Healthcare West (CHO 2) (ddpo.west@hse.ie)

Date:

Re: Cross-Divisional Request (Acute & Mental Health) – UCHG & CHO 2

Dear Data Protection Officer(s),

I am writing to formally make a **Subject Access Request (SAR)** under **Article 15 of the GDPR** and the **Data Protection Act 2018 (Ireland)**.

1. Dual-Targeting Strategy (Saolta & CHO 2)

I am aware that Mental Health Services at University Hospital Galway (UCHG) are administratively managed by **Community Healthcare West (CHO 2)**, while the hospital infrastructure and Emergency Department are managed by the **Saolta University Health Care Group**.

- **Instruction:** This request is directed to **BOTH** controllers. You are legally obliged under the GDPR's cooperation principle to coordinate to locate my **"Psychiatric" file** (held by the Mental Health division) and my **"Medical/ED" file** (held by Saolta). Do not close this request by claiming "no records held" without checking both divisions.

2. Legal Basis: Reasonable Accommodation (Equal Status Acts 2000–2018)

I am a person with a disability, specifically **Traumatic Brain Injury (TBI)** and **C-PTSD**.

- **The "Nano Nagle" Precedent:** As established by the Supreme Court in *Nano Nagle School v Daly*, service providers must take "appropriate measures" to accommodate

disabilities unless it causes a disproportionate burden.

- **The Burden Argument:** Providing me with paper records (which I cannot cognitively process due to TBI) when digital formats exist is a failure to accommodate. Providing a digital export from your **IPMS** or **NIMIS** systems is not a disproportionate burden; it is a standard function of modern IT.
- **Requirement:** I require all data in **digital, searchable formats**.

3. Scope of Request

A. Psychiatric & Mental Health Records (CHO 2):

- All "Kardex" nursing notes, medical notes, and consultant psychiatrist entries. If these exist only in handwritten form, I request **high-resolution scans processed with OCR**.
- **Involuntary Admission Forms:** Copies of any applications or orders made under the **Mental Health Act 2001** (specifically Forms 1 through 6) related to any involuntary admission.
- **Tusla Correspondence:** Any reports, referrals, or correspondence sent to or received from **Tusla** (Child and Family Agency) regarding me.

B. Radiology & Imaging (NIMIS):

- I request a copy of all imaging (MRI/CT/X-Ray) held on the **NIMIS** PACS system.
- **Format:** Please provide these as **DICOM** files via a secure digital transfer or on a CD/USB. Do **not** provide paper prints of X-rays, as they are diagnostically useless and fail the "Reasonable Accommodation" standard.

C. Emergency Department (Saolta):

- All Triage notes, ED clinical notes, and discharge summaries from my attendances at UCHG.

4. Nominated Health Professional (S.I. No. 121 of 2022)

Under the **Data Protection Act 2018 (Access Modification) (Health) Regulations 2022**, if you believe access to my mental health data would cause "serious harm," you must not merely refuse access.

- **Nomination:** I hereby preemptively nominate **Dr. [Name]** (Details below) to receive any such withheld data.
- **Proxy Details:**
 - Name: Dr. [Name]
 - Address: [Address]
 - Email: [Email]
- **Note:** Failure to release the data to me **OR** my proxy will be considered a breach of Regulation 9 of the Statutory Instrument and will be reported to the Data Protection Commission.

5. Verification

Please find attached a **Solicitor-Certified Copy of my Passport** to verify my identity from London.

I expect a response within one calendar month as per statutory timelines.

Yours sincerely,

[Address]

[Phone]

Conclusion

The barriers to access presented by large health trusts—fragmented departments, "paper-only" legacy policies, and broad "serious harm" exemptions—are formidable but surmountable. By framing the request through the lens of **Disability Rights (Reasonable Accommodation)** and **Technical Competence (Epic/NIMIS Forensics)**, the Data Subject shifts the burden of proof back onto the institutions.

They must now explain why they cannot press the "Export" button in Epic or NIMIS, rather than the Data Subject explaining why they need the data. The "Nominated Health Professional" strategy acts as the ultimate failsafe: even if the institutions successfully argue that the records are too dangerous for the student to see, the legal obligation to release them to the nominated doctor ensures the evidence is secured for the defense.

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